Challenge Stigma | Find Peace

FrontlineFolks | Guide #30

When Stress Shuts Down Sex: High Alert, High Cortisol, and Trouble Getting or Finishing

---

A Field Guide for Frontline Folks and Their Families

Erectile dysfunction and anorgasmia are nervous system problems, not character flaws.

By Michael Jenkins, MS, MSW, LCSW
Never Forgotten Services, PLLC

Disclaimer

This guide is for general education only.

It is not:

- Therapy
- Medical care
- A crisis service
- Individualized advice

Reading this guide does not create a therapist–client relationship with the author or with Never Forgotten Services, PLLC.

If you are in crisis or thinking about harming yourself or someone else, call 988 in the United States, go to your nearest emergency room, or contact your local emergency services.

If you want mental health treatment, please contact a licensed provider in your state or call the number on the back of your insurance card.

Table of Contents

1. What Is Happening
2. Why It Escalates
3. What To Do This Week
4. For Partners and Team
5. When To Get More Help
6. 7-Day Field Drill
7. Worksheets
    - Worksheet 1: Worksheet: When Stress Shuts Down Sex: High Alert, High Cortisol, and Trouble Getti
    - Worksheet 2: Worksheet: When Stress Shuts Down Sex: High Alert, High Cortisol, and Trouble Getti

1. What Is Happening

You can't get or stay hard. Or you come too fast. Or you can't come at all. Or you lose the erection mid-sex. These aren't character flaws—they're nervous system problems. High arousal state (hypervigilance) prevents the parasympathetic activation needed for erections and orgasm. High cortisol from chronic stress suppresses sexual response. Hyperarousal sensitizes the nervous system so you lose erection from small triggers or come before you want to. The more you panic about it, the worse it gets. Anxiety about performance becomes the thing that kills performance.

Common sexual dysfunction patterns in frontline folks

- Difficulty getting or maintaining erections: The nervous system is too activated for parasympathetic response.
- Rapid ejaculation: Nervous system is sensitized; orgasm happens before arousal builds enough.
- Delayed ejaculation or anorgasmia: Hypervigilance prevents letting go enough to orgasm.
- Loss of erection during sex: Small triggers (partner sound, change in position, intrusive thought) kill arousal.
- Difficulty with desire leading to performance: You're not interested, then you're anxious about not being interested, then you can't perform.
- Performance anxiety: Panic about losing erection creates the exact conditions that cause it.
- Avoidance of initiating: Fear of failure starts keeping you from even trying.

> **This is not sexual failure:**
> This is a dysregulated nervous system interfering with the parasympathetic activation required for sexual response. It's fixable.

2. Why It Escalates

Sex requires parasympathetic activation: calm, relaxation, trust, blood flow to genitals. Hypervigilance prevents all of this. The sympathetic nervous system (fight-flight) is incompatible with sexual arousal. Frontline work keeps you in sympathetic mode. Your body is literally saying 'we're under threat, not safe to be vulnerable.' Add performance anxiety, and you're fighting your own nervous system. Cortisol suppresses sexual response. Sleep deprivation worsens everything. Depression and trauma both interfere with sexual function.

How stress kills sexual function

- Sympathetic dominance: Hypervigilance prevents the parasympathetic calm needed for arousal.
- High cortisol: Cortisol blocks sexual hormone production and suppresses arousal.
- Hyperactive startle: Small triggers (sound, touch intensity, position change) activate threat response and kill arousal.
- Hyperarousal sensitivity: Nervous system is so sensitized that any stimulus triggers orgasm response too early.
- Sleep deprivation: Poor sleep worsens erectile function and arousal speed.
- Performance anxiety: Worrying about erection creates the exact stress that causes erectile difficulties.
- Avoidance: After failures, you stop initiating, which creates more anxiety and more failures.
- Dissociation during sex: Hypervigilance makes you check out mentally, preventing full arousal and orgasm.

3. What To Do This Week

Sexual dysfunction is treatable. You need nervous system downshift first, then specific sexual attention work.

1. Reduce whole-system activation: Movement, breathing work, sleep, stress reduction all improve sexual function.
2. Build parasympathetic activation routines: Slow breathing, warm baths, meditation, time in nature—all activate rest-and-digest.
3. Remove performance pressure entirely: Stop trying to have sex. Zero goal to have an erection or orgasm.
4. Rebuild arousal capacity: Start with non-sexual touch and physical affection only. No intercourse. Rebuild arousal slowly.
5. Practice 'start-stop' technique for rapid ejaculation: Pause when orgasm is approaching, let arousal drop slightly, then continue. Retrains nervous system.
6. Use mindfulness during sex: Staying present and anchored prevents dissociation and maintains arousal.
7. Address performance anxiety directly: 'My body had a stress response. That's normal. I'm rebuilding trust with my body.'
8. Consider medical evaluation: Low testosterone or other hormonal issues might be contributing; testing can clarify.
9. Get sex therapy: Specialized treatment for erectile dysfunction and anorgasmia exists and works.

> Field rule: set the bar low enough to do it even on your worst day.

4. For Partners and Team

Stress communication should be clear, brief, and repeatable. Predictable routines protect relationships.

- Use concrete language: 'I'm having trouble because my nervous system is too activated. This isn't about you or my attraction.'
- Agree on zero-pressure sex: Sex is off the table. Affection is allowed. Rebuilding happens gradually.
- Debrief without blame: 'My body's doing what it learned to do at work. I'm working on retraining it.'
- Consent, pacing, and emotional safety are part of intimacy, not separate from it: Partner can't pressure this back to normal. It comes back through safety.

---

5. When To Get More Help

Self-guided tools are a start. Escalate care when symptoms persist, spread, or begin harming safety and function.

- Sexual dysfunction has persisted 6+ months despite stress reduction and sleep recovery.
- Avoidance is expanding: You're avoiding partner, avoiding intimacy, isolating more.
- Performance anxiety is severe: Panic before or during sex, catastrophizing about function.
- Relationship conflict about sex is escalating.
- Sex therapy or trauma-informed couples care specialized in sexual dysfunction can accelerate recovery and rebuild confidence.

6. 7-Day Field Drill

Use the worksheet every day for one week. Track trigger, body signal, and reset action. Keep what works and remove what does not.

> **Daily drill:**
> 1) Name the trigger.
> 2) Name the first body signal.
> 3) Run one reset action.
> 4) Log response quality.

Worksheet 1: Worksheet: When Stress Shuts Down Sex: High Alert, High Cortisol, and Trouble Getting or Finishing

Use this to track the pattern in this chapter and build a practical response you can repeat under load.

Where did this pattern show up this week?

List specific moments at work, at home, and in transition.

What did it cost you when it took over?

Name impact on body, mood, decisions, and relationships.

What two actions will you run for the next 7 days?

Choose actions small enough to use on hard days.

1. Trigger:

What activated the pattern?

2. Early body signal:

What did your body do first?

3. Reset action used:

What fast action lowered intensity?

4. Better next response:

What exact behavior will you use next time?

> Track this daily for one week, then tighten the plan around what actually worked.

Worksheet 2: Worksheet: When Stress Shuts Down Sex: High Alert, High Cortisol, and Trouble Getting or Finishing - Partner/Team Check-In

Use this page for one short practical check-in each week.

What do others notice first when this pattern is active?

Capture observable behavior, not labels.

What support helps most and least?

List what helps and what escalates the issue.

What agreement will you both practice this week?

One clear action from each side.

1. Signal phrase:

What phrase means pause and reset?

2. Time-to-return commitment:

When will re-engagement happen?

3. Repair step:

What specific repair happens after a rupture?

> Keep check-ins short, specific, and repeatable.

Closing

You've seen and carried more than most people will ever know.

Blow ups and shutdowns are not proof that you're broken or that your relationship is doomed.

They are signs that your nervous system has been running in survival mode for a long time.

The fact that you picked up this guide and read it matters.

You are choosing to be intentional about how you show up for the people who matter to you.

That choice is a lot more powerful than having flawless days.

About the Author

Michael Jenkins, MS, MSW, LCSW

Founder, Never Forgotten Services, PLLC

Michael is a U.S. Army combat veteran and licensed clinical social worker who specializes in working with military, veterans, and first responders—our frontline folks. After leaving the military, he spent years fighting his own demons and trying to figure out why he could hold it together on the job but feel completely wrecked at home. The turning point was realizing he wasn't the only one living that split life—and that nobody should have to feel that alone, confused, or broken by what the job asks of them.

Since then, his work has focused on trauma, the impact of high-stress jobs on home life, and helping families make sense of what the job brings home. He uses plain language, zero shame, and a "you're not crazy, this actually makes sense" approach to help frontline folks and their families understand what's happening in their nervous systems and learn how to move through the chaos together.